

## Behavioral Emergency and De-escalation (Code Gray)

|                |                                    |                    |                                                                                                         |
|----------------|------------------------------------|--------------------|---------------------------------------------------------------------------------------------------------|
| Policy Number  | POL-PC-061                         | Owner / Department | Nursing / Security                                                                                      |
| Effective Date | 2023-09-15                         | Version            | 2.1                                                                                                     |
| Last Revised   | 2026-02-22                         | Next Review        | 2028-02-22                                                                                              |
| Approved By    | Patient Safety & Quality Committee | Applies To         | All staff in all patient-care and public areas; coordinated with Security and the Administrator-on-Call |

### 1. Purpose

To provide a safe, coordinated response to behavioral emergencies that emphasizes early recognition and verbal de-escalation, reserving physical intervention for imminent danger, consistent with patient rights under CMS Conditions of Participation (42 CFR 482.13).

### 2. Scope

Applies to any patient, visitor, or situation involving escalating agitation or threatening behavior that risks the safety of the patient, staff, or others, in any area of the facility.

### 3. Definitions

- **Behavioral emergency** — an acute disturbance of behavior, thought, or mood that poses an imminent risk of harm and requires an immediate response.
- **Code Gray** — the facility overhead alert summoning a trained behavioral-response team and Security for a combative or threatening person.
- **De-escalation** — verbal and nonverbal techniques used to reduce agitation and restore the person's ability to engage, without physical force.

### 4. De-escalation Approach

- Maintain a calm demeanor and a safe distance; keep an unobstructed exit for yourself and the patient.
- Use a single spokesperson, simple language, and a respectful tone; acknowledge feelings and offer realistic choices.
- Reduce environmental stimulation; remove potential weapons and bystanders from the area.
- Set clear, non-threatening limits and allow time for the person to respond.

### 5. Code Gray Response

**IMPORTANT** — For any imminent threat of harm, call a Code Gray and Security immediately and ensure staff and bystander safety first. De-escalation is always attempted first; physical intervention is a last resort. Restraint or seclusion for a behavioral emergency is governed by POL-NUR-005 and requires an order and a 1-hour face-to-face evaluation.

1. Call a Code Gray; summon Security and the behavioral-response team.
2. Clear the immediate area of other patients and visitors; assign a single spokesperson to lead verbal de-escalation.
3. Attempt de-escalation; offer voluntary medication and support as clinically appropriate.
4. If imminent danger persists despite de-escalation, apply restraint or seclusion only per POL-NUR-005.
5. After the event, provide medical evaluation for anyone injured, debrief the team and patient, and file an event report per POL-RM-013.

## 6. Severity & Reporting

Events are classified and reported under POL-RM-013. A behavioral emergency causing serious injury to a patient, staff member, or visitor, or any death associated with restraint or seclusion, is SEV-1 — notify the attending provider, Charge Nurse, Nursing Supervisor, Risk Management, and the Administrator-on-Call immediately and within 1 hour. An assault or injury requiring treatment is SEV-2, reported within 4 hours. A contained episode with no injury is SEV-3 or a near miss, reported within 24 hours. Staff injury from patient or visitor violence is additionally handled under the workplace-violence policy.

## 7. Documentation Requirements

- Trigger and behavior observed; de-escalation techniques attempted and the response.
- Whether a Code Gray was called and who responded.
- Any medication offered or given, and any restraint or seclusion applied (cross-reference POL-NUR-005).
- Injuries, notifications, and the post-event debrief.

## 8. References

- CMS Conditions of Participation, 42 CFR 482.13 — Patients' Rights.
- The Joint Commission, Provision of Care standards on behavioral management and de-escalation.
- Related: POL-NUR-005 (restraint & seclusion), POL-PC-060 (suicide risk), POL-EH-011 (workplace violence), POL-RM-013 (severity & reporting).